Health Technology Ecosystem

ID.me Response to

Centers for Medicare & Medicaid Services (CMS)
RFI for Feedback on Digital Health Products,
Data Interoperability, &
Broader Health Technology Infrastructure

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June 16, 2025

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Executive Summary

The Problem

Healthcare costs in the U.S. are projected to reach $5 trillion annually by the end of 2026, according to CMS.[1] A significant portion of this cost stems from inefficiencies related to fraud, lack of interoperability, information blocking, and poor data quality. These systemic challenges hinder the ability to deliver effective, efficient care and must be addressed to support Secretary Kennedy's initiative to "Make America Healthy Again." The Secretary has stated that he "will **provide Americans with transparency and access to all the data** so they can **make informed choices for themselves and their families.**"[2]

How federated, Portable Digital Identity can help

A **federated digital identity solution offers a powerful approach to unlocking transparency and access to data** — especially if it is grounded in TEFCA, OMB M-19-17, and National Institute of Standards & Technology Special Publication (NIST SP) 800-63-3, Digital Identity Guidelines (AAL2/IAL2). Across 20 federal agencies, 46 state agencies, and over 30 EHR's, ID.me has consistently shown the benefits of this model. We are the longest-tenured holder of Kantara certification, the only FedRAMP authorized provider to offer three (3) pathways to verification, and have already credentialed more than 74 Million Americans to NIST AAL2/IAL2 standards. These "preverified" users can simply login with ID.me, complete MFA, and consent to release their existing credential to a new agency, with pass rates of 99%+. Compared to algorithm-only solutions, ID.me has doubled access rates, generated hundreds of millions in operational efficiencies, and played a critical role in preventing $273 billion in fraudulent unemployment insurance payments during the pandemic.[3]

The IRS, the VA, and SSA currently rely on federated identity models that include ID.me and Login.gov as credential service providers (CSPs). If CMS were to follow suit – and demonstrate the benefits to the rest of the healthcare industry – the benefits of the federated identity approach would compound for providers and patients alike. For example, a beneficiary that uses an ID.me credential at Social Security to begin the application for Medicare could use that same credential to manage benefits at Medicare.gov, The beneficiary experiences less friction and CMS would have more certainty that the person creating a Medicare.gov account is in fact the same individual that Social Security determined to be eligible for Medicare. This approach eliminates friction for millions of existing credential-holders and strengthens data quality and record matching across agencies. In regard to providers, ID.me protects the National Provider Data Bank at HRSA, the National Board of Medical Examiners, and 30% of the nation's electronic prescriptions.

Benefits for CMS, Medicare, and Medicaid

When leveraging ID.me, CMS will significantly enhance online access and transition from a reactive "pay and chase" model to a proactive "prevent and detect" framework. ID.me's user-centric digital wallet model is already in-use across federal, state, and private sector healthcare entities and has successfully participated in pilot programs under HHS and the CARIN Alliance.

Incorporating the ID.me login button on the CMS beneficiary and provider-facing websites enables these 74 Million pre-verified users to seamlessly share their credentials without additional verification steps. This integration delivers a streamlined user experience, enhances security for both users and CMS, accelerates system deployment, improves data quality, and enables robust identity management while

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[1] CMS. (2022, March 28). *CMS Office of the Actuary Releases 2021-2030 Projections of National Health Expenditures* [Press release]. http://www.cms.gov/newsroom/press-releases/cms-office-actuary-releases-2021-2030-projections-national-health-expenditures.

[2] https://govciomedia.com/trumps-hhs-secretary-pick-eyes-transparency-data-access/

[3] (2022, November 17). ID.me Statement on House Oversight Committee Statement. *PR Newswire*. https://www.prnewswire.com/news-releases/idme-statements-on-house-oversight-committee-statement-301682275.html.

leveraging a deterministic matching model to ensure the right person has access to the right records at the right time.

By addressing identity concerns upfront, CMS can allocate resources to other critical development priorities, ensuring a more efficient and secure operational framework.

Recommendations

ID.me's immediate and specific recommendations to CMS are to:

1. **Encourage sector-wide adoption of portable, NIST-conformant, FedRAMP authorized digital credentials by adopting them at CMS and demonstrating the benefits.** SSA (an entry point for Medicare) and the VA (the largest integrated health system in the U.S.) both offer access via federated identities: ID.me and Login.gov. More than 74M citizens are already using ID.me to access these agencies and their records. Re-use of these credentials at Medicare.gov and Healthcare.gov would increase American's access to their health records, reduce onboarding friction, increase security, and improve care quality.

2. **Require CSPs to deploy advanced fraud controls capable of stopping emerging threats from generative AI (e.g. deepfakes).** NIST 800-63-3 is an effective standard, but it is also a ten year old standard. Threats from generative AI are evolving faster than federal guidelines can, and so it is up to CSPs to stay ahead of emerging threats through biometrics and their own AI.

3. **Require capabilities beyond NIST conformance that are critical to managing business user applications: ability to manage delegated access, and have a unified identity graph with continuous trust monitoring.** CMS, payers, and providers interact with a broad range of personas and relationships: patients, providers, clinical staff, caretakers, and plan managers. CSPs that can support delegated and relationship-based access control models will enable more secure transactions across personas.

4. **Update Interoperability & Patient Access Rule.** ID.me recommends updating the Interoperability and Patient Access Rule to mandate use of third-party credentials that meet federal policies and assurance standards like the Office of Management and Budget (OMB) Memorandum 19-17[4], NIST SP 800-63, and NIST 800-53 (i.e. FedRAMP).

[4] "M-19-17 Enabling Mission Delivery through Improved Identity, Credential, and Access Management." Digital.gov, 6 Sept. 2023, digital.gov/resources/m-19-17-enabling-mission-delivery-through-improved-identity-credential-and-access-management. Accessed 13 June 2025.

Requested Feedback

Patients & Caregivers: Information Blocking & Digital Identity

Managing multiple logins has always been frustrating. With fraud and generative Al scams on the rise, the internet is increasingly less safe. ID.me solves both problems with a secure and convenient login you can use across any website you visit to easily and safely prove your identity. No more passwords to manage. We're making life safer and easier.

Consumer-centric, portable credentials are becoming more commonplace, and the U.S. healthcare sector has an amazing opportunity to benefit from their use. Of the 260 million adult citizens in the U.S., 148 million users have ID.me accounts, and 74 million of them are verified to a NIST high level of assurance – that's more than a quarter of all American adults. The primary difference between these credentials and what has historically been used across the healthcare industry is that these credentials "travel" with the end-user across organizations, rather than being specific and captive to a single enterprise. By orienting access and credentials around the patient rather than the enterprise, CMS can **deliver on the Secretary's aspiration for patients and their caregivers to have easier access to their medical records, enabling more awareness and quality of care.**

The largest integrated health system in the U.S. – the Department of Veterans Affairs (VA) – **has been using portable digital identity credentials for nearly a decade**.[5] 8.4 million VA beneficiaries already have portable credentials and use them to manage their healthcare: providing consent to sharing their medical records, booking appointments, communicating with their doctors, and ordering prescription refills. As of September 30th of this year, VA will only accept portable credentials, after it removes DS Logon credentials as a login option.[6]

Another example of how beneficiary-centric digital identities are being used effectively at-scale is at the Social Security Administration (SSA), which serves as an entry point and the eligibility pathway for Medicare. **As of this month, SSA transitioned to a 'full federation' user experience on SSA.gov:** "Effective June 7, 2025, Login.gov and ID.me are now the only sign in options to use Social Security's online services. [The SSA has] removed the option to sign in using a Social Security username and password."[7] They now only accept beneficiary-centric, portable digital credentials. This also means that any user Medicare beneficiary that initiates enrollment from SSA.gov will have a portable digital identity.

Now that these "installed bases" of portable credentials have been established and are available for use, CMS – and the rest of the healthcare industry – could leverage this strategic investment to capture clear benefits: better patient experience, higher patient access rates, lower fraud, and operational efficiencies. CMS could get started by simply adding ID.me and Login.gov as login options at Medicare.gov. This would allow users that created ID.me or Login.gov credentials at SSA.gov as part of their Medicare journey to then use those same credentials to manage their benefits at Medicare.gov.

Today, a citizen is able to log into the SSA, using their ID.me credential, and apply for eligibility for Medicare. This "re-use" reduces the number of registrations a user has to complete and reduces the time and effort expended by the beneficiary. It would also help with deterministic record matching across agency back-end systems between CMS and SSA.

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[5] *VA Works with Veteran-Run Company ID.me to Offer Veterans More Secure, Convenient Online Services*. (2016, December 7). Veterans Affairs. Retrieved June 13, 2025, from https://news.va.gov/press-room/va-works-with-veteran-run-company-id-me-to-offer-veterans-more-secure-convenient-online-services/

[6] *Prepare for VA's secure sign-in changes* (2025, March 5). Veterans Affairs. Retrieved June 13, 2025, from https://www.va.gov/initiatives/prepare-for-vas-secure-sign-in-changes/

[7] *Create an Account | my Social Security | SSA*. (n.d.). Retrieved June 13, 2025, from https://www.ssa.gov/myaccount/create.html

Both the VA and SSA took the approach they did – offering Login.gov and ID.me – because they see benefits of offering beneficiaries multiple options: system resilience and availability, increased consumer choice, innovation through competition, and greater user control over their journey. As a result, at the VA, each month, 1.4 million Veterans use the app to manage their VA services, refill prescriptions, message their health care team, check the status of claims, manage appointments and more. The VA app has a 4.8 star rating in the Apple app store, with over 200,000 ratings, and is one of the most successful digital experience products in the federal government.

This type of model could easily be extended out to the broader healthcare sector by adopting a standardized, patient-centric approach to patient portals. If all patients and providers were to adopt a unified user experience where patients could use their ID.me or other portable credentials (and Login.gov for government programs), then providers and payers would only ever be interacting with patients that were (1) verified to NIST levels of assurance, (2) resolved to a single unique identity, and (3) had additional attributes by which records could be matched and de-duplicated. This approach to identity would form the cornerstone of data quality as records because records would always be attached to a single, unique individual.

Think about it like a Visa card. Once you get a Visa payment credential from any credential issuer, you can use it anywhere Visa is accepted. The way healthcare operates today is like the way department stores used to operate: each department store in the mall offered its own credit card, and you could only use that card at the one store. Therefore, if you needed to buy goods at three (3) different stores, you needed three (3) different cards. It was a terrible experience for the consumer.

Today, nearly every merchant accepts Visa, Mastercard, and American Express, and it's up to the consumer to decide which card they want to use based on the benefits of using those cards, the service levels the consumers receive, and their trust in the card issuer.

Mallory Schroder, a U.S. citizen and patient, does a great job of describing this pain point in her personal response to this Federal Register/RFI:

> "A consistent frustration I've had as a patient is having multiple logins for various clinics, rather than one that contains all of my records in one place. Most people don't go to the same doctor their whole lives. They jump from pediatrician to maybe a doctor in college, then attempt to find primary care doctors in new locations as their career progresses. This has been my experience. Throughout this I've had to manage records through pdfs that I'm attempting to store and provide to new healthcare providers, rather than this information being available to me in one consistent portal. Perhaps there's already a solution to this, but none that is broadly advertised or explained to me by a clinic."

Portable credentials are the lowest-risk, highest-benefit way for CMS and the entire healthcare sector to make it easier for patients like Mallory to access their medical records and consent to sharing them as they receive care from different providers and payers.

PC-14. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800–63–3 IAL2/AAL2 credentialing service providers (CSP)):

a. What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials?

Unpacking Ms. Schroeder's statement leads us to four (4) challenges with the current system:

1. **Friction in the User Experience**. Patients are asked to manage multiple credentials across different services, complete redundant verification steps, and navigate inconsistent login experiences. This often proves especially difficult for those who are less comfortable with technology, those with limited digital literacy, and communities historically excluded by traditional verification methods.
2. **Technical Barriers to Access.** Legacy verification methods powered by credit bureaus often struggled to verify the CFPB-reported 45 million Americans with thin credit files. Legacy algorithm-only methods also leave behind individuals facing housing instability, name changes, or living overseas. Offering alternative pathways like video chat or in-person verification using diverse documentation significantly increases access and pass rates for these underserved populations. By *not* transitioning to modern, portable credentials, these users are subject to repeating the same antiquated process at each new provider, health system, or payer they interact with.
3. **Trust and Privacy.** A digital identity credential from a CSP like ID.me gives patients control over their data. They can audit what information is shared with whom at any time by visiting account.id.me. With a portable, federated credential, users can manage, update, or revoke their information through a single account, enhancing both privacy and trust.
4. **Ecosystem Barriers to Adoption.** Proprietary EMR systems often block external IdPs, forcing patients to manage multiple logins and repeatedly verify their identity. CMS can lead the way by requiring support for standards-based federation with NIST-compliant CSPs, unlocking secure, seamless identity reuse across the healthcare ecosystem.

When it comes to adoption of portable digital credentials, the biggest challenge is end-user awareness. Many individuals only begin to learn about what these credentials mean for them when they first "encounter" one at a website protected by a CSP. Organizations we work with overcome this challenge through very proactive communications campaigns where they raise awareness via multiple channels:

- Agency blogs
- Agency email newsletters
- Updates to copy on websites
- Links to help articles about CSPs, secure authentication, and identity proofing
- Content distributed through advocacy or support organizations

b. What could be the benefits to patients/caregivers if digital identity credentials were more widely used?

In accordance with the 21st Century Cures Act, patients should be given:

- **Increased access:** ID.me's digital identity credentials are designed to be user-centric, streamlining enrollment and logins at the variety of different healthcare services a patient may require. This includes hard-to-reach users as the ID.me Digital Wallet offers multiple pathways to complete verification and integrated capabilities for login, MFA, identity proofing, and access management. In the event a user does not possess the required identity evidence to complete online, self-service proofing at AAL2/IAL2, ID.me offers Supervised Remote (video chat) verification and in person verification. An example of this is at a current large healthcare agency who is reporting access rates at better than 2x for all users and 3x for those considered to be "underserved" such as those with no US Federal ID, those without bank accounts, etc.
- **Increased usage of digital services:** One ID.me customer has reported a usage rate of almost 2x of what was expected (50k users per month is now 80k users per month). The reason for this increase has been attributed to easier access due to pre-verification, ease of use, and faster response times for logging in.

- **Reduced Reduction in information blocking**. According to 2017 Pew research, 1 in 5 medical records is not matched within the same healthcare system and up to 50% of records are not matched in transfers. Gregg Church, President of 4medica, estimates that 30% of patient records – over 100 million nationwide – are duplicates.[8] This problem is exacerbated by information blocking and concerns over liability associated with transmitting incorrect records or transmitting records to the wrong individuals. This will continue unless CMS calls for strong, deterministic digital identity.
    - Federated digital identity can reduce information blocking by providing two key elements:
        - **A longitudinal history of attributes** (e.g. previous addresses and names), which is critical for resolution of records and continuity of care
        - A **deterministic view of the patient** in which truly unique identifiers deliver a definitive match to a patient's identity
    - Public-private partnerships (e.g., CARIN Alliance and Sequoia Project) have demonstrated effective solutions that can resolve records to a single unique individual. Providing a centralized and verified source for provider credentials.
    - Streamlining and automating the credentialing and data verification processes.
    - Enhancing secure and efficient data exchange between healthcare stakeholders.
    - Supporting the creation of longitudinal histories of provider attributes, which is crucial for continuity of care and quality improvement.
- **Reduced administrative burden:** After replacing a legacy white-label identity provider and a leading data broker, the Virginia Employment Commission (VEC) saw digital claims increased by 173% while call center volume dropped by 57%, easing operational burden on both the claimant and VEC. VEC Commissioner Demetrios Melis said, "This new login option solves the main challenge we've been dealing with for some time, providing our customers with easy access while keeping out fraud. By partnering with ID.me, we are able to do just that."[9] (Listen to his full testimony to the Virginia General Assembly here.) The transformation was so impactful it inspired the award-nominated "It's EZ with ID.me" campaign—highlighting a simpler, more secure way for the public to access government services.[10]
- **Enhanced privacy:** ID.me will only share data with the informed consent of the end user, on a case-by-case basis. **ID.me does not sell data.** This first-party informed consent model is why NIST awarded ID.me more than five (5) million dollars in grant funding back in 2013 to advance our solution across the market.

[8] Miliard, Mike. "4medica to Help HIE Tackle Patient Data Challenges in East Tennessee." *Healthcare IT News,* 6 Dec. 2021, https://www.healthcareitnews.com/news/4medica-help-hie-tackle-patient-data-challenges-east-tennessee.
[9] ID.me for Government: The Identity Layer for Digital Government Services. Vimeo, uploaded by ID.me, 9 May 2024, https://vimeo.com/1027116236. Accessed June 13, 2025.
[10] "It's E-Z with ID.me: Virginia Employment Commission Offers Faster and Secure Login Option – Unemployment." *Virginia Employment Commission*, 7 Mar. 2024, https://www.vec.virginia.gov/node/18403. Accessed June 13, 2025.

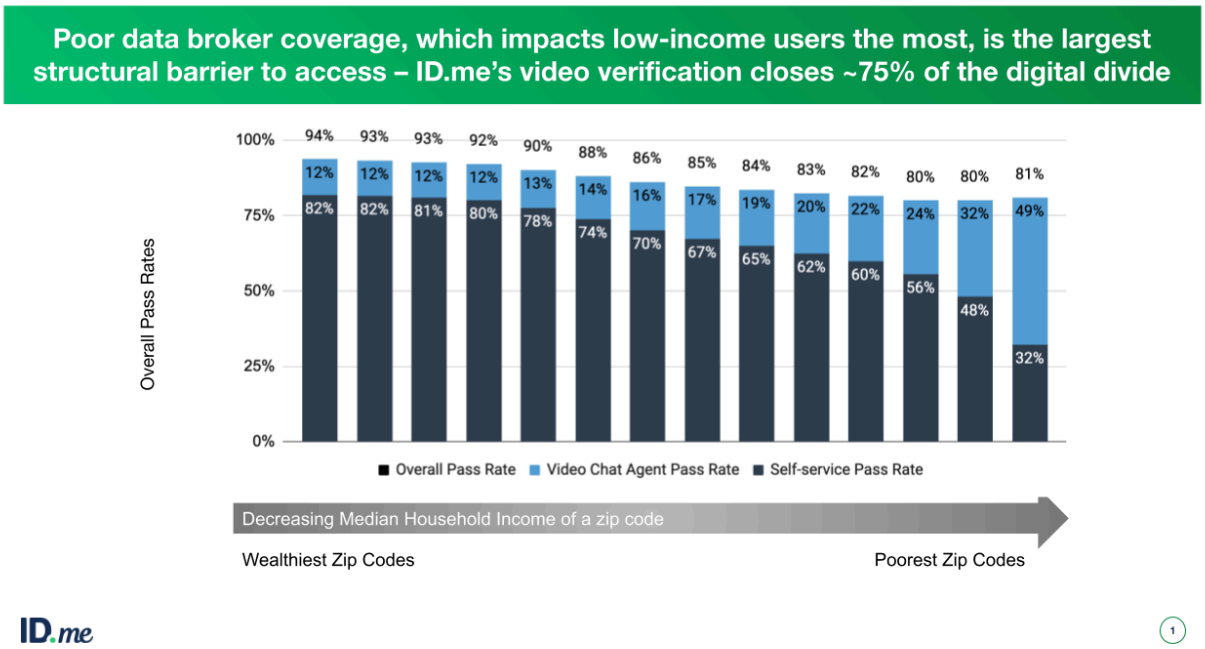

Figure 1: ID.me's identity orchestration platform allows us to see the individuals who cannot pass credit bureau and data broker flows as we can verify them via video chat and in-person

c. What are the potential downsides?

Digital identity credentials offer clear benefits. The biggest potential downside is if CMS does not "hold a high bar" on what CSPs should be used by payers and providers. CMS should only recommend full-service CSPs or "digital wallets" which (1) are certified against NIST 800-63 and NIST 800-53 (i.e. FedRAMP), (2) are privacy-enhancing and do not sell user data, (3) offer omnichannel verification to ensure maximum access but equitable access to healthcare for hard to reach populations, and (4) offer 24/7 user support for the full scope of credential authentication issues.

- **Higher Security.** Sensitive personal and health information could be exposed if systems aren't up to current security standards. CMS should only recommend CSPs that demonstrate a commitment to security with the following:
    - ***Certifications:*** CSPs should maintain compliance with FedRAMP moderate ATO, NIST 800-63-3 AAL2/IAL2, SOC 2 Type II, and ISO 27001
    - ***Adaptive Authentication:*** Invoking device verification on authentication attempts across agencies and organizations, plus post-verification fraud controls, maintains the patient's credential security over time, in alignment with Zero Trust Architecture.
- **Enhanced Privacy.** Patients may fear that increased digital tracking could lead to misuse of personal data, such as being shared with third parties (e.g., insurers, advertisers) without consent. CSPs backed by CMS can help promote patient privacy by implementing:
    - ***First Party Consent***: Data brokers build extensive user profiles by buying and scraping data without that user's awareness or consent. CMS and other healthcare organizations should instead look for vendors like ID.me who follow a First Party Consent model and only gather and release data upon consent by the authenticated user.
    - ***Data Minimization***: This approach means the CSP shares only collect and share the necessary attributes for each transaction. This model stands in sharp contrast to data

brokers, who build extensive user profiles without that user's awareness or consent by scraping public data sources and social media websites.
    - ***User Control of Data:*** CSPs that are committed to privacy will empower patients to manage, update, revoke, or delete their data at any time via a single account page tied to a portable, federated credential. ID.me users can log into account.id.me at any time to see what data fields ID.me has shared with what organizations, update information, and delete their account. Every other CSP used by CMS should be expected to provide the same level of transparency.
    - ***Commitment to not selling user data*. ID.me does not sell user data, under any circumstance.** Users are solely in control of their own data and must provide consent before any information is shared.
- **Greater access for underserved populations.** If CMS does not hold a high bar for credentials that offer multiple pathways for verification, it could lead to the unintentional exclusion of groups without access to smartphones, broadband, or adequate support. Patients should have the ability to verify their identity via video chat or in person at staffed kiosks as a fallback to online self-service verification. Verification and authentication flows should also be available in multiple languages.
- **Reduced Account Administration Burden.** Credentials need maintenance over time. Users need to reset their authenticators, move addresses, or change names. ID.me offers 24/7 end-user support from domestically-based agents to help users with the full scope of account lifecycle needs. Users can access support via multiple channels, including online ticket submission, chatbot, live chat, and video calls. CMS should expect CSPs to deliver a similar level of service for end users.

d. How would encouraging the use of CSPs improve access to health information?

Leading CSPs can improve both access to health information and the quality of that information.

- **Access to information**. ID.me has been shown to increase access to online services by 2x relative to algorithm-only solutions and 3x for underserved populations. Once access is granted, adoption of digital services increases. We expect the same to be true for beneficiaries and patients viewing health information and managing care, as we see at the VA today.

    - **Quality of the information**. CSPs can improve interoperability by reducing record duplication and fragmentation. Portable NIST credentials can do this because they eliminate reliance on probabilistic matching and replace it with deterministic matching of records. Deterministic matching occurs when truly unique identifiers deliver a definitive match to a patient's identity, validated by their real-time control of the digital session—through methods like biometrics or live video—and enforced within a single-tenant identity system to ensure the identity hasn't already been claimed. This reduces risk of HIPAA violations in response to records queries because it is a binary "yes/no" for the record match rather than a sliding score. This improved record matching and easier transfer of records means patients will have more complete clinical information aggregated into a single record.

For AAL2/IAL2 identity proofing, ID.me enforces uniqueness per NIST guidelines for identity resolution by assigning a Unique Identifier (UUID) to each account based on Name, Date of Birth (DOB), and Social Security Number (or another unique identifier, such as a passport or national ID number). This "rule of three" ensures a 1:1 relationship between a user and their ID.me UUID, preventing fraud and unauthorized access. Once a user verifies at IAL2 and adds an AAL2 passkey, **no one else can claim their identity.**

The UUID is included in the identity payload returned to organizations after successful verification or authentication. Organizations can store the UUID for future authentication and as an attribute in the user's profile, for example in an IAM network's universal directory, enabling a unified applicant view across systems. This allows healthcare organizations to accurately match patient data and track service usage, while also maintaining security and privacy.

e. What role should CMS/payers, providers, and app developers have in driving adoption?

In order to promote adoption, CMS should:

1. Integrate CSPs into its guidelines for accessing federal health data (e.g., Blue Button, Medicaid portals)
2. Mandate AAL2/IAL2, which would deliver a unique identifier for each individual and improve query results
3. Lead by example by adopting federated digital identity across its external-facing portals

Payers, providers, and app developers can drive adoption and reduce fragmentation by:

1. Accepting portable credentials as part of their login processes. Payers, providers, and app developers should integrate federated authentication into all relevant flows, e.g., patient portals, telehealth platforms, and in-person check-in workflows, and collaborate with HIEs and other providers to align on shared identity standards. This fosters familiarity and trust and increases pass rates, while reducing the number of passwords patients need to remember.
2. Educate patients and caregivers about the benefits of using CSPs – amplifying any messaging that originates from CMS. Providing this background upon or ahead of rollout will contribute faster adoption from patients.
3. Commit to adherence to the Web Content Accessibility Guidelines (WCAG) 2.1 / 2.2 Level AA, the Americans with Disabilities Act (ADA), and Section 508 of the Rehabilitation Act, in order to support increased access. Adoption improves when users are not required to understand identity standards but simply benefit from their existence.

f. How can CMS encourage patients to get digital identity credentials?

By following the example set by the VA. They communicated frequently with end-users about the benefits of adopting digital identity credentials. They also engaged the broader ecosystem that supports patients: medical centers, MyHealtheVet coordinators, local VA chapters, Veteran Support Organizations (VSOs), and state VA programs. It was a coordinated campaign across functions where they drove a comprehensive change management plan that included time-phased transitions where the VA would accept all credentials prior to accepting federated ones. The VA helped patients understand that changes were being made to benefit them: better access to health information, reduced administrative burden of managing their records, easier management of care, and increased the security of their health information.

Providers: Digital Identity

Similar to how digital credentials can simplify access to patient portals, ID.me also streamlines care delivery and medical record workflows for medical professionals and the clinical staff that support them. So when CMS considers adoption of digital identity by "Providers", we encourage CMS to consider two lenses: (1) the clinics and hospitals that provide care, and (2) the medical professionals that work there.

For the clinics and hospitals, adopting federated digital identity unlocks the patient-facing benefits we described in the previous section. It also unlocks benefits for the clinics and hospitals themselves:

- ***Increased adoption of digital services:*** With higher access rates and a familiar login experience, federated digital identities from ID.me have been shown to increase adoption of digital services. This eases the burden on in-person administrative activities at hospitals and clinics. At the Virginia Employment Commission, claims submitted digitally increased by 173% vs. paper submissions once ID.me was offered as a login option on the claims portal.
- ***Better fraud Prevention:*** A shared authentication service across providers can see how threat actors are targeting multiple providers using the same stolen identities and block fraudulent users' access to all partner agencies. All providers that leverage ID.me would benefit from fraud signals that are aggregated from across its network of government agencies and private sector entities. Seven (7) workforce agencies are on the record crediting ID.me with helping to prevent over $273 billion dollars in fraud during the pandemic.
- ***Improved Data Quality.*** CSPs support interoperability by enforcing uniqueness per NIST guidelines for identity resolution and supporting standards-based authentication, making cross-provider data sharing more efficient and more accurate. Additionally, identity-linked access logs enhance auditability, helping to improve compliance, oversight, and trust in digital health infrastructure.
- ***Lower Total Cost of Ownership:*** CSPs like ID.me handle all login, MFA, and verification support requests. Organizations that adopt ID.me typically see a 7-10x return on investment (ROI) within three (3) years, largely driven by reductions in call center demand, integration costs, and reduced fraud losses. A provider should expect the total cost of ownership (TCO) for federated digital identity to be ⅓ to ½ what it is with the point solution tech stacks widely used today.

The biggest thing CMS can do to advance adoption of digital credentials by providers is to focus on the large EMRs. A majority of hospital systems offer patient portals powered by the major EMRs. Each of these portals is built in the brand or style of the individual Providers. That can still hold true; however, the login experience for patients needs to be consistent across sites so that patients know they can use the credentials they know and trust from other sites (similar to seeing a Visa or Paypal logo when checking out of a retail website). Some of the EMRs have begun to leverage federated digital identities, but are only doing so at a single point in time: verification as part of the onboarding process. These EMRs then require patients to use non-federated credentials for ongoing authentication activity. Because the EMR credentials are captive to that EMR's systems, this approach does not capture the full benefits of federated digital identities and erodes patient experience as they move between provider networks and clinics. **CMS can set the example of what they want EMRs to make available to Providers by showing them "what good looks like" on its own patient and medical professional facing portals.**

For medical professionals, federated digital identity streamlines access to workflows across care sites, EHRs, and government portals in a way that is aligned with NIST guidelines and HIPAA requirements. **Today, there are more than 3.7M medical professionals in the ID.me network.** These professionals can add additional attributes – such as their National Provider Index (NPI) or Drug Enforcement Agency (DEA) registration – to their ID.me wallet. These additional attributes, which are bound to

NIST-conformant identity credentials, are used to support clinical workflows such as electronic prescriptions.

Medical professionals consent to ID.me sharing their identity and other attributes to perform a broad range of clinical, claims, and records management workflows. **Today, 30% of the country's electronic prescriptions leverage ID.me credentials.** Our scale and experience adding additional attributes to ID.me digital wallets sets us apart from other CSPs. ID.me's relationship with medical professionals starts from the beginning of their professional journey and spans:

- **Registering for US Medical Licensing Examination** (USMLE) Step exams by completing NIST IAL2 proofing with ID.me.
- Requesting and **managing NPI numbers** at HRSA's National Practitioner Data Bank
- **Submitting electronic prescriptions** for both uncontrolled (eRx) and controlled substances (EPCS) by completing NIST-compliant IAL2 remote identity proofing and AAL2 multi-factor authentication, alongside NPI and DEA registration
- Submitting **Medicare Part A and B appeal requests** in the Office of Medicare Hearings and Appeals (OMHA) e-Appeal Portal
- Completing medical **record transfer requests via TEFCA** interoperability framework (see article here)
- Submitting **claims for care delivered to veterans** via the CHAMP VA and eCAMS applications on VA.gov
- **Managing referrals for all Community Care providers** through the VA's HealthShare Referral Manager
- **Submitting Medicaid claims** and **verifying Medicaid providers** at state portals
- Submitting **state disability insurance claims**
- Reporting cancer **clinical trial treatment data** to the National Cancer Institute
- **Uploading clinical research** to NIH's All of Us Research Program (AoURP)

As CMS is looking to improve care administration via a nationwide provider directory, **ID.me can already deliver a user base of medical professionals that would be included in and may require access to the directory.** While CMS looks at data sources by which to build it (e.g. health plans submit information on which providers are authorized to provide which services), digital identity will be critical to linking ingested information to the unique providers. Providers could log in, authorize ingestion of their records, and view and manage their records. CMS can get started by simply adding ID.me as a login option at its provider-facing portals, such as the CMS.gov Enterprise Portal, HARP, and PECOS.

PR–9. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

CMS can encourage providers to accept portable credentials from patients and their partners by:

1. **Making policy changes:**
    a. Issue a final rule requiring any provider who delivers care under a CMS-funded program (e.g., Medicare Advantage, Medicaid MCOs) to accept digital identities from certified IdPs that meet NIST 800-63-3 assurance levels and FedRAMP ATO.
2. **Offering technical solutions and proof of effectiveness:**
    a. Launch CMS-supported pilots (possibly under Innovation Center (CMMI) initiatives) with health systems to demonstrate cost, security, and access benefits of accepting federated identities.

   b. Offer a CMS or HHS built credential broker to minimize integration work by smaller hospitals and clinics. For example, offering providers a choice of integrating with CSP's directly or integrating with HHS XMS and deploying a Use Case ID such that CMS can determine the volume of usage by different clinics and hospitals
3. **Providing business case justifications and technical guidance:**
   a. Provide case studies and ROI models showing how accepting verified digital credentials reduces friction, prevents fraud, and supports better digital engagement with members.
4. **Deploying a combination of incentives and protections:**
   a. Subsidize provider use of digital identity for the first 2 years so long as it is done in accordance with CMS best practices and guidance (e.g. offering CSPs as a login vs. a one-time verification behind a payer authentication process)
   b. Create goals related to adoption of digital identity in STAR ratings such that higher use of digital credentials results in higher scores and higher payments for care delivered

a. What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?

The two biggest things CMS can do is (1) show Providers "what good looks like" by offering federated digital identity on its provider-facing portals and (2) focus on the large EMRs.

Some of the EMRs have begun to leverage federated digital identities, but are only doing so at a single point in time: verification as part of the onboarding process. Because the EMR credentials are captive to that EMR's systems, this approach does not capture the full benefits of federated digital identities and erodes patient experience as they move between provider networks and clinics. CMS should Select a large EMR for pilot purposes to show patients and providers what streamlined identity can bring.

There are smaller, more tactical activities that CMS can take to ensure a smooth transition:

- **Technical Readiness.** Provide playbooks and reference material on standards such as FHIR, OAuth 2.0 or OpenID Connect, and SMART on FHIR to enable secure, consistent data exchange.
- **Address liability concerns.** Providers may have concerns about HIPAA compliance when responding to TEFCA queries. CMS should listen to those concerns and alleviate them by showing how digital identity – when done right – can reduce the risk of response errors.
- **Change management and patient acceptance.** CMS can help Providers by speaking directly to their patients and the public about the benefits of federated digital identity. CMS should communicate often, openly, and in multiple channels about how this transition **will create easier, more secure access to health information and enable lower costs for care.**

b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?

To balance patient privacy with convenient access to digital health services CMS should require CSPs to follow best practices rooted in NIST 800-63A recommendations for user control of data.[11] These practices are also consistent with major privacy regimes such as the California Consumer Privacy Act (CCPA) and the Illinois Biometric Information Privacy Act (BIPA):

- ***First Party Consent***: Data brokers build extensive user profiles by buying and scraping data without that user's awareness or consent. CMS and other healthcare organizations should instead look for vendors like ID.me who follow a First Party Consent model and only gather and release data upon consent by the authenticated user.

[11] https://nvlpubs.nist.gov/nistpubs/SpecialPublications/NIST.SP.800-63a.pdf

- ***Data Minimization***: This approach means the CSP shares only collect and share the necessary attributes for each transaction. This model stands in sharp contrast to data brokers, who build extensive user profiles without that user's awareness or consent by scraping public data sources and social media websites.
- ***User Control of Data:*** CSPs that are committed to privacy will empower patients to manage, update, revoke, or delete their data at any time via a single account page tied to a portable, federated credential. ID.me users can log into account.id.me at any time to see what data fields ID.me has shared with what organizations, update information, and delete their account. Every other CSP used by CMS should be expected to provide the same level of transparency.
- ***Commitment to not selling user data*. ID.me does not sell user data.** Users are solely in control of their own data and must provide consent before any information is shared.

PR–10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800–63–3 IAL2/AAL2 CSPs):

a. What are the challenges and benefits for providers?

The biggest challenge to providers is going to be the EMR's. As discussed above, the EMRs need to make federated credentials available as login options when building and offering patient portals to Providers. CMS can set the example of what they want EMRs to make available to Providers by showing them "what good looks like" on its own patient and medical professional facing portals such as Medicare.gov, Data at the Point of Care, and the Enterprise Portal.

Beyond the benefits described above, providers would also receive:

- ***Out-of-the-box Compliance:*** Providers that adopt ID.me would have a solution that holds certifications against SOC 2 Type II, ISO 27001, FedRAMP Moderate ATO, and NIST 800-63-3 AAL2/IAL2. ID.me is also compliant with major privacy regimes such as California's CCPA and Illinois' BIPA.
- ***Zero Trust Architecture (ZTA) Alignment****:* ID.me offers continuous authentication and risk-based access controls, which enable interoperability across organizations while maintaining security. This Zero Trust approach ensures continuous fraud checks, not just at account creation, but at every critical interaction, such as subsequent logins to the CMS portal. ID.me supports a comprehensive set of MFA capabilities at AAL2 and AAL3 to ensure security for high-risk use cases and demographic accessibility. With each MFA event, ID.me supports a device fingerprinting solution that looks across ID.me's shared services network to detect if a device is used for fraud or identity theft. These capabilities augment MFA at no extra cost.

b. How would requiring their use improve access to health information?

1. **Interoperability and Trusted, Streamlined Data Exchange:** Requiring verified digital credentials helps standardize identity across all systems, making it easier to plug into national frameworks like TEFCA and collaborate with providers and third-party apps.
2. **Better Access to Longitudinal Health Data:** By enabling seamless, federated access, providers can use a single secure login to retrieve patient data from multiple organizations, promoting data quality and minimizing mistakes or delay in treatment.
3. **Faster, More Secure Authorization Workflows.** Digital identity credentials allow for real-time identity verification during data exchange requests, accelerating prior authorizations, appeals, and patient-directed data sharing.
4. **Greater Member Engagement.** When patients and providers can use a single digital credential to access multiple payer and provider systems, they're more likely to engage with their health

records, claims data, and benefits. Greater engagement leads to better member satisfaction and retention, and improved adherence to care plans. Today roughly 65% of authentications across ID.me's network involve a user simply logging in and consenting to release their verified identity versus new IAL2 verifications. We estimate that by the end of 2025, over 80% of authentications across our network will be users who have already verified before coming to one of ID.me's partner organizations.

c. What are the potential downsides?

Smaller providers and clinics will need access to inexpensive, off-the-shelf systems that large EMRs may not be willing to provide. CMS can support small providers by offering subsidies or an HHS-run broker that smaller clinics can leverage (e.g. HHS XMS with the ability to track use cases down to the individual provider).

d. What impact would mandatory credentials have on a nationwide provider directory?

Implementing strong digital identity proofing significantly enhances the accuracy and reliability of provider directories. Every provider in the directory would be tied to a federated, verified digital credential, eliminating outdated, duplicate, or fraudulent entries. This creates a single source of truth for identity, increasing confidence in health data and in who is accessing and sharing it.

With mandatory credentials, directory entries can be reliably linked to FHIR API endpoints and identity attributes, enabling secure, automated data exchange across systems. Tying credentials to verified demographic and licensure information also enables automated updates, allowing changes in provider status, location, or role to be reflected in real time, upon each authentication event, without manual intervention. This reduces the risk of duplicate or outdated listings, ultimately improving the overall trustworthiness and utility of healthcare directories.

Mandating AAL2/IAL2 credentials will help ensure all health data exchanges start with verified participants and known endpoints. These credentials provide a foundational trust layer that enables TEFCA to function securely, ensures USCDI data can be reliably attributed to the correct provider, and helps CMS enables CMS to monitor, measure, and enforce compliance more effectively, since credentialed identities can be tied to usage logs and access policies.

e. How could digital identity implementation improve provider data flow?

Digital identity credentials enable more efficient, secure, and transparent data exchange within and across healthcare systems. By requiring authentication to access patient data, CMS can ensure that each data request or submission comes from a verified doctor, nurse, or clinical staff, eliminating ambiguity and reducing the risk of errors, fraud, or unauthorized access. This strengthens point-to-point trust between systems (e.g., between EHRs, payers, and HIEs), leading to faster approvals, referrals, and data sharing.

With a unique, verified identity attached to every medical professional, health data entries (e.g., clinical notes, orders, referrals) are clearly attributable to the correct individual, supporting clinical accuracy, billing integrity, and audit compliance. At scale, these credentials eliminate the need for manual vetting or outdated methods like fax-based record requests, enhancing both speed and security.

f. Would combining FHIR addresses and identity improve data flow?

Combining FHIR addresses (FHIR API endpoints) with verified digital identity credentials would significantly improve data flow in healthcare by enabling secure, precise, and automated interoperability

between trusted parties. Today FHIR addresses identify **where** to send or retrieve data, but without knowing **who** is sending, receiving, and viewing the data, systems must rely on additional checks or risk unauthorized access. Pairing a FHIR endpoint with a verified NIST 800-63-3 AAL2/IAL2 credential ensures that only authenticated individuals or systems can access the endpoint, enabling trusted, direct exchange. By linking a provider's digital credential to their specific FHIR endpoint, systems can enable targeted data delivery—routing information directly to the intended recipient rather than a general organizational inbox.

The pairing of identity and FHIR address also simplifies directory matching, making participant lookups more accurate and efficient. Furthermore, identity-bound endpoints support event-driven exchange, allowing real-time updates such as lab results or ADT events to be delivered automatically, without the need for manual routing or intervention.

PR–11. How could members of trust communities 3 (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security?

Members of trust communities—such as QHINs, Participants, and Subparticipants under TEFCA—are well-positioned to reduce administrative burdens on both providers and patients while strengthening identity management and security. TEFCA already mandates Identity Assurance Level 2 (IAL2) through CSPs. However, to truly scale and enhance usability, these entities must move beyond basic compliance and proactively collaborate to standardize and streamline identity workflows.

One key strategy is promoting federated and reusable identities within the TEFCA ecosystem and leveraging protocols like OpenID Connect for identity federation across organizations. This reduces credential duplication and the need for repeated identity proofing. Standardizing identity workflows through common implementation profiles, aligned with NIST 800-63-3 and ONC-endorsed standards like FHIR and OAuth 2.0, can lower integration friction and embed security by design. Implementing “authenticate once, use everywhere” models enables providers and patients to use a single verified session across the network, improving user experience and reducing repetitive logins.

Shared governance frameworks—including audit trails, incident response protocols, and joint monitoring—help build trust and detect misuse. Collaborative education efforts across QHINs can boost digital literacy, reduce training overhead, and help both patients and providers navigate digital credentials effectively.

Payers

The benefits described above for patients and providers compound if payers were also to take a similar approach. Two issues we hear from Payers is that (1) access rates are lower than they want and (2) there is low participant adoption of the services Payers offer participants online. As discussed above, portable digital credentials increase access and drive adoption of digital services. Federated digital identities will unlock operational efficiencies for plans and payers just as they have for government agencies.

As previously described under **Question PC-14**, earlier this year, the VEC made ID.me a login option on the claims portal. When they did, they saw a 173% increase in claims submitted online (vs. paper) and a 57% decrease in demand into their call center. In discussions with other major health plans, they cite that success rates accessing online services are only 55% and that only 40% of their enrollees engage in services online. This proves the point that the greater the friction to the end user, the greater the likelihood that patients will turn away from online services and, in turn, drive costs up for payers.

In contrast, the largest integrated health system in the US – Veteran Affairs – acts as both a payer and a provider. Just this week, VA CTO Charles Worthington reported that *each month* "1.4 million Veterans use the app to manage their VA services, refill prescriptions, message their health care team, check the status of claims, manage appointments and more." **That is 16% of all enrolled patients engaging online *each month***. Access to the app is enabled via federated digital identities and leverages secure authentication like passkeys. New features related to Payer activities are "coming soon", such as expanded access to medical records and the ability to file travel claims for care directly on the app.

Payers benefit from federated digital identity because it enables secure, standards-based digital identity verification and access control, which enhances data interoperability, fraud prevention, and regulatory compliance while providing increased and accurate customer service. Specifically:

1. **Improved data quality and reduced data exposure risk:** Payers require access to clinical data in HIEs for use cases like prior authorization, care coordination, risk adjustment, and quality measurement. NIST-conformant CSPs like ID.me enforce uniqueness per NIST guidelines for identity resolution and supporting standards-based authentication, making data access more efficient and more accurate. ID.me ensures the payer is working with the correct patient, helping to reduce data quality issues and risk of sharing sensitive information with the wrong person.

2. **Increased online access for participants:** ID.me offers higher access rates versus point solutions powered by credit bureaus and data brokers. Additionally, Payers would see fewer disruptions related to lifecycle management of the credentials. Patients may switch jobs and payers or may have multiple payers throughout their lives. Patients and Payers will spend less time updating their information and managing the life cycle of the credentials because that can be done once with the CSP and then transmitted automatically to Payers during the next authentication event.

3. **Lower Total Cost of Ownership*:*** CSPs like ID.me handle all login, MFA, and verification support requests. Organizations that adopt ID.me typically see a 7-10x return on investment (ROI) within three (3) years, largely driven by reductions in call center demand, integration costs, and reduced fraud losses. A Payer should expect the total cost of ownership (TCO) for federated digital identity to be ⅓ to ½ what it is with the point solution tech stacks widely used today.

4. **Audit Trails and Compliance Support:** Patients expect secure access to their Protected Health Information (PHI) and payers must demonstrate secure access to PHI for compliance with HIPAA, HITECH, and state-level privacy laws. ID.me offers detailed audit logs, access

attestation, and user traceability for payer employees accessing health records. This simplifies audits and enhances transparency in payer access to HIEs. **ID.me does not handle, process, or store PHI** and is therefore technically not subject to HIPAA; however, ID.me SaaS has been architected to comply with HIPAA.

5. **Enabling TEFCA-Compliant Data Exchange:** Under TEFCA, payers participating as QHINs or subparticipants must adhere to rigorous identity and access management requirements. ID.me provides a compliant identity layer that supports the TEFCA technical framework and data-sharing standards. This helps to facilitate seamless, standards-aligned data exchange between payers and providers through HIEs and QHINs.

As a starting point to unlocking these benefits, CMS – as a Payer itself – can set the example by adopting federated digital identity on its beneficiary-facing sites such as Medicare.gov.

PA–1. What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?

- **Ambiguity around identity resolution standards.** TEFCA requires identity proofing at IAL2 for certain access scenarios (e.g. individuals accessing or transferring their records) but does not prescribe uniform standards for matching patient records across QHINs. This leads to variability in how identity is resolved and opens the door for inconsistent patient matching success.
- **Recommendation:** TEFCA should improve patient matching accuracy by adopting standardized identity assertion schemas (e.g., inclusion of verified attributes like phone, email, or hashed identifiers). Additionally, given the fact that there are 45 million people in American that are "thin file" and will struggle to verify their identity using algorithm-only solutions, TEFCA can increase the number of records and individuals able to engage in TEFCA by requiring use of CSPs with proven fallback pathways for underserved populations (e.g., video chat for individuals that don't appear in data brokers and credit bureaus).
- **Impact on Participation:** These limitations, if unaddressed, could significantly deter participation from digital health innovators, consumer app developers, and identity platforms—slowing adoption among forward-leaning providers and payers who rely on scalable, standards-based integrations. Moreover, entities serving vulnerable populations may find participation challenging if TEFCA does not adopt solutions that can onboard hard-to-reach populations and maximize inclusion.

PA–2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

CMS can accelerate adoption of API-based data exchange by continuing to lead by example—not only by mandating interoperability, but by role-modeling workflows built on digital identity. Blue Button 2.0 and Data at the Point of Care (DPC) were powerful first steps, showing that standardized, FHIR-based APIs can make Medicare data securely accessible to patients and providers. Now, CMS can extend that leadership by building the next generation of patient onboarding and care coordination workflows—starting with trusted identity.

For example, imagine a payer enrolling a new member and using the TEFCA Individual Access Services (IAS) framework, which calls for NIST AAL2/IAL2 credentials, to query for the patient's longitudinal health record. Instead of relying solely on self-reported history or claims lag, the plan could retrieve a FHIR Patient resource and linked data from across TEFCA-connected providers at the point of enrollment

—lab results, problem lists, medication histories. With that data, the plan could instantly surface care gaps, determine preventive screening schedules, and personalize medication monitoring, creating a frictionless, data-rich onboarding experience that improves care from day one.

To make this a reality, CMS should not only require API support from payers, but also:

- Incentivize identity integration with TEFCA IAS and Patient Access APIs using NIST 800-63-3 compliant credentials.
- Demonstrate real-world workflows that connect identity, consent, and FHIR-based data exchange in live CMS programs.
- Collaborate with private payers to pilot these experiences, using CMS as the early adopter and proving ground for scalable models.

By setting the standard for how identity, consent, and data exchange come together to enable personalized, proactive care, CMS can turn policy into practice and create the momentum necessary for ecosystem-wide adoption.

PA–3. How can CMS encourage payers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins?

CMS can encourage payers to accept portable credentials from patients and their partners by:

1. **Deploying a combination of incentives and protections:**
    a. Include adoption of digital identity in scoring metrics for STAR ratings, so that plans that have high adoption receive bonus payments
    b. Reducing the liability risk for HIEs to respond to TEFCA queries, so long as they:
        i. Adopt the recommended identity assertion schemes
        ii. Use CSPs that have been independently assessed to conform with NIST 800-63 guidelines by the Kantara Initiative or other CMS-approved certification body
        iii. Use CSPs that have FedRAMP authorization to operate
    c. Subsidize payer use of digital identity for the first 2 years so long as it is done in accordance with CMS best practices and guidance (e.g. offering CSPs as a login vs. a one-time verification behind a payer authentication process)
    d. Create goals related to adoption of digital identity in Value-Based Care and Administrative Simplification such that higher use of digital credentials results in higher scores and higher payments for care delivered
2. **Making policy changes:**
    a. Issue a final rule requiring any plan that is participating in CMS-funded programs (e.g., Medicare Advantage, Medicaid MCOs) to accept digital identities from certified IdPs that meet NIST 800-63-3 assurance levels and FedRAMP ATO.
3. **Offering technical solutions and proof of effectiveness:**
    a. Launch CMS-supported pilots (possibly under Innovation Center (CMMI) initiatives) with major payers to demonstrate the cost, security, and access benefits of accepting federated identities.
    b. Offer payers use of a CMS- or HHS-built credential broker to minimize the integration work and engineering changes required by payers on their patient-facing portals. An example of this could be offering a payer the choice of integrating with CSP's directly or integrating with HHS XMS (and deploying a Use Case ID such that CMS can determine the volume of usage of credentials by specific payers).

Providing business case justifications and technical guidance:

1. Provide case studies and ROI models showing how accepting verified digital credentials reduces friction, prevents fraud, and supports better digital engagement with members;
2. Build and deploy implementation guides that show payers how to integrate with trusted identity providers using FHIR, OpenID Connect, and OAuth 2.0 standards.

PA–4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

A nationwide provider directory that includes FHIR endpoints and is linked to verified digital identity credentials would offer substantial value to payers across administrative efficiency, care coordination, compliance, and interoperability.

1. **Enhanced Data Interoperability and Network Discovery**
   a. **More accurate provider records**. With FHIR endpoints integrated into a nationwide directory, payers can identify and query provider systems to retrieve clinical data (e.g., Continuity of Care Documents, lab results). This eliminates reliance on manual directories, fax-based data exchange, or outdated point-to-point connections.
   b. **Improved patient experience**. Payers can surface real-time, accurate provider information in member portals or customer service tools, helping members find in-network or interoperable providers more easily.
2. **Stronger Trust and Security via Verified Digital Identities**
   a. **Reduced potential for fraud.** Linking each provider in the directory to a NIST AAL2/IAL2-verified digital identity reduces fraud risk (e.g., false NPIs, billing scams).
   b. **Simplified Credentialing and Access Control**. Payers can use digital credentials to authenticate providers for secure APIs, portals, and communication, rather than issuing proprietary logins.
3. **Streamlined Administrative and Clinical Workflows, such as Prior Authorization and Record Retrieval**
   a. **Accelerated Prior Authorization and Record Retrieval**. A payer needing documentation for utilization review or care management can query the directory for a provider's FHIR endpoint and initiate a standards-based data pull, authenticated by the provider's digital credential.
   b. **Support for Risk Adjustment and Quality Reporting**. Payers conducting risk scoring or HEDIS audits can use the directory to retrieve supplemental clinical data from provider systems securely and efficiently.
4. **Reduced Fraud, Waste, and Abuse**
   a. **Better detection of fraud**. With verified digital credentials, payers can detect anomalous provider behavior, flag access attempts from spoofed or unverified identities, and better enforce role-based access to patient data. Combating phantom billing, identity fraud, and network leakage becomes more feasible when directory data is both identity-verified and interoperable.
   b. **Operational efficiencies.** Early adopters can reduce IT overhead by not managing their own directories, improve provider satisfaction (via seamless data exchange), and differentiate on digital readiness. As value-based care accelerates, such a directory positions payers to be better collaborators in care coordination, data sharing, and network optimization.
5. **Compliance and Alignment with Federal Mandates**
   a. **Supports TEFCA, Patient Access Rule, and Interoperability Goals**. The directory would enable payers to participate in trusted health information networks (QHINs) by offering a scalable, identity-assured lookup mechanism aligned with FHIR standards. It also supports CMS Patient Access and Provider Directory API requirements, offering a nationwide source of truth that reduces redundant infrastructure.

Conclusion

ID.me applauds CMS and HHS for its efforts to simplify patient access to healthcare information. Based on our experience supporting 148 million digital wallets, 20 federal agencies, 45+ state agencies, and over 500 private sector entities on with our portable digital identities, we recommend that CMS:

1. Encourage sector-wide adoption of portable, NIST-conformant, FedRAMP authorized digital credentials as a login option by adopting them at CMS and demonstrating the benefits.

2. Require CSPs to deploy advanced fraud controls capable of stopping emerging threats from generative AI (e.g. deepfakes).

3. Require capabilities beyond NIST conformance that are critical to managing business user applications: ability to manage delegated access, and have a unified identity graph with continuous trust monitoring.

4. Update the Interoperability & Patient Access Rule to mandate use of third-party credentials that meet federal policies and assurance standards like the Office of Management and Budget (OMB) Memorandum 19-17, NIST SP 800-63, and NIST 800-53 (i.e. FedRAMP).

Leveraging ID.me's proven infrastructure, CMS can **give patients greater access to their health information by dramatically reducing onboarding friction and mitigating fraud risk. In addition to improved access, patients, payers, providers, and EMR's will also have better quality health information as digital identity can reduce duplication and fragmentation of records as they move across networks.** We are eager to meet with CMS to continue this discussion and align on how to "Make America Healthy Again."